

This report is designed to test an AI's ability to recognize oncological patterns through symptom clusters, specific imaging terminology, and constitutional signs, without using the word "tumor" or "cancer."

PATIENT MEDICAL REPORT: SPECIALIST EVALUATION

1. PATIENT INFORMATION

- **Name:** Margaret H. Whitmore
- **Age:** 62
- **Gender:** Female
- **Date of Visit:** January 14, 2026
- **Medical Record Number (MRN):** 3390-PL-4481

2. CHIEF COMPLAINT & SYMPTOMS

- **Primary Symptoms:** Persistent, dull localized pain in the right upper quadrant (RUQ) and a noticeable, firm "fullness" upon palpation.
- **Duration and Severity:** Symptoms have progressed over 4 months. Pain is now a constant 6/10, worsening after meals.
- **Associated Symptoms:** Unintentional weight loss of 18 lbs in 10 weeks. Early satiety (feeling full quickly), profound fatigue (malaise), and recent onset of mild scleral icterus (yellowing of the eyes).

3. MEDICAL & FAMILY HISTORY

- **Past Medical Conditions:** Chronic Hepatitis C (treated), Non-alcoholic fatty liver disease (NAFLD).
- **Current Medications:** Multivitamins, Omeprazole 20mg (for acid reflux), and occasional Ibuprofen for RUQ pain.
- **Allergies:** Sulfa drugs (Reaction: Rash).
- **Family History:** Father deceased age 65 (Lynch Syndrome positive); Mother deceased age 58 (Malignant process of the colon); Brother currently undergoing "intensive systemic infusion therapy" for a growth in the lungs.

4. PHYSICAL EXAMINATION

- **Vital Signs:**
 - **BP:** 118/74 mmHg
 - **HR:** 82 bpm
 - **Temp:** 37.4°C (99.3°F)
 - **RR:** 18 breaths/min
- **Physical Findings:** Patient appears cachectic (muscle wasting). Abdominal exam reveals a palpable, non-mobile mass extending 4cm below the right

costal margin. The mass is irregular in shape and firm to the touch. No rebound tenderness noted.

5. LABORATORY RESULTS

Test	Result	Normal Range	Status
Hemoglobin	10.1 g/dL	12.0 - 15.5 g/dL	Low (Anemia)
AST / ALT	145 / 160 U/L	10 - 40 U/L	Elevated
Alkaline Phosphatase	410 U/L	44 - 147 U/L	High
Bilirubin (Total)	2.8 mg/dL	0.1 - 1.2 mg/dL	High
Alpha-Fetoprotein (AFP)	550 ng/mL	< 10 ng/mL	CRITICAL
CEA Level	12.5 ng/mL	< 3.0 ng/mL	Elevated

6. IMAGING STUDIES

- **Contrast-Enhanced CT (Abdomen/Pelvis):** A 5.2 cm heterogeneous, hypervascular lesion is identified within Segment VII of the hepatic lobe. The lesion demonstrates "washout" in the venous phase.
- **MRI with Gadolinium:** Confirms a solid, space-occupying lesion with irregular borders. There is evidence of localized lymphadenopathy (enlarged nodes) in the porta hepatis. No evidence of free fluid (ascites) at this time.
- **Chest X-Ray:** Presence of multiple small, well-circumscribed "coin lesions" bilaterally in the lower lobes.

7. CLINICAL NOTES

Assessment: 62-year-old female with a high-risk family history presenting with cachexia and RUQ pain. Laboratory markers (specifically the AFP and CEA) are

significantly out of range, suggesting a highly active proliferative process. Imaging confirms a primary solid focal lesion with secondary evidence of distal involvement in the pulmonary fields.

Observations: The "washout" pattern seen on CT is highly characteristic of specific cell-type growths in this organ. The patient's rapid weight loss and anemia suggest a high metabolic demand from the lesion.

Plan:

1. CT-guided core needle biopsy of the hepatic lesion.
2. Referral to Surgical Oncology and Palliative Care for symptom management.
3. PET scan for full-body staging.

Attending Physician: Dr. R. Vance, MD

Department: Oncology / Hepatology

Would you like me to generate a report for a **Neurological** condition (like a brain-related growth) or perhaps a **Respiratory** issue to see if the AI can tell the difference?